

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part One — The Architecture Alberta Built and the Lifecycle Failures It Produced

Section 1.7 — Mental Health and Addiction as a Lifecycle Dimension

Sections 1.1 through 1.6 documented the architecture Alberta has built around persons with disabilities and the lifecycle dimensions across which the architecture operates: childhood and FSCD (Section 1.3), the engineered repudiation of inclusion in the K-12 system (Section 1.3a), the transition into PDD and AISH at adulthood (Section 1.4), the caregiver dimension across lifecycle stages (Section 1.5), and the triage question that the WCB exception raises (Section 1.6). The lifecycle frame established in Section 1.2 is the substantive analytic structure within which the documented failures are most clearly visible. Section 1.7 develops a substantive lifecycle dimension that the prior sections do not anchor: mental health disability, addiction, and the substantive intersection of the two within the Alberta architecture.

The dimension is operationally significant. Mental illness disorders constitute the second-largest primary medical-condition category among AISH recipients, accounting for thirty-and-two-tenths percent of the September 2025 caseload of seventy-nine thousand two hundred ninety AISH clients (twenty-three thousand nine hundred forty Albertans), behind only physical disabilities at forty-one-and-five-tenths percent and ahead of cognitive disorders at twenty-eight-and-three-tenths percent. Source: Alberta Assisted Living and Social Services, Caseload by Primary Medical Condition for September 2025, available at <https://open.alberta.ca/dataset/df7b7b00-c5c3-4d0e-97ff-5f902537fac0>. The AISH-to-ADAP transition documented in Section 1.4 affects this population in operationally specific ways the prior sections do not address. The substantive Alberta record on mental health disability includes the involuntary-confinement provisions of the *Mental Health Act* of Alberta, the documented inadequacy of community-based mental health supports, the AISH-eligibility complications produced by mental-health-disability profiles, and the documented intersection of mental health disability with homelessness and incarceration. The substantive Alberta record on addiction includes the documented opioid-and-stimulant toxicity crisis, the documented provincial-government shift from harm-reduction to recovery-focused policy, and the substantive *Compassionate Intervention Act* (Bill 53, Royal Assent May 15, 2025) that operationalizes involuntary addiction treatment within the substantive provincial framework. The two records intersect substantively: the populations the Alberta architecture characterizes as mental-health-disability and as addiction are substantially overlapping, and the substantive policy responses to the two records operate within an integrated provincial framework.

The section develops the substantive content in seven components. The first establishes the substantive distinction between mental illness as a temporary medical condition and mental health disability as a sustained dimension of social participation. The second documents how mental health and addiction operate across the lifecycle dimensions the prior sections established. The third develops the substantive Alberta record on mental health disability. The fourth develops the substantive Alberta record on addiction and substance use. The fifth addresses the *Compassionate Intervention Act* and the involuntary-treatment framework as the substantive operational expression of the provincial-government's

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

mental-health-and-addiction policy direction. The sixth addresses the substantive co-occurrence of mental health and addiction within the population the legislation is intended to serve. The seventh synthesizes what the section establishes for the substantive case the blueprint develops.

The substantive content of Section 1.7 is grounded in primary-source documentation: provincial-government open data on AISH composition, peer-reviewed academic literature on Alberta mental-health disability, the Public Health Agency of Canada national surveillance data on opioid- and stimulant-related harms, the Alberta Substance Use Surveillance System data on Alberta-specific drug-poisoning deaths, and the substantive professional-and-academic engagement with the *Compassionate Intervention Act* including the Canadian Medical Association Journal ethical analysis, the Alberta Medical Association response, and the Canadian Bar Association Alberta legal analysis.

The Substantive Distinction: Mental Health Disability and Mental Illness

The substantive distinction between mental illness as a medical condition and mental health disability as a dimension of social participation is operationally significant for the substantive case the blueprint develops. The medical-model framing characterizes mental illness as a condition the individual *has* — a diagnosis, a set of symptoms, a treatment trajectory operating within the healthcare system. The social-model framing characterizes mental health disability as the substantive interaction between the individual's mental-health profile and the barriers the social environment imposes — the substantive disability is in the interaction, not in the diagnosis alone. The distinction matters because the substantive policy responses to mental health under the two framings are operationally distinct: the medical-model framing produces clinical-treatment-oriented policy responses focused on symptom management and individual rehabilitation; the social-model framing produces accessibility-and-inclusion-oriented policy responses focused on barrier removal and substantive participation.

The distinction is operationally significant for the AISH-eligibility framework. AISH eligibility, as documented in Section 1.4, requires a "severe and permanent" medical condition that "substantially limits the ability to earn a living." The substantive operational test for mental health disability under this framework is medical-model in its substantive design: a qualified medical professional must certify that the mental-health condition meets the severity-and-permanence threshold. The substantive operational reality is that mental health disability frequently operates episodically rather than continuously, fluctuates over time, responds variably to treatment interventions, and may be characterized differently by different qualified medical professionals depending on the substantive professional context within which the assessment is conducted. The medical-model AISH-eligibility framework accordingly produces structural barriers to access for persons with substantive mental-health-disability profiles whose disability does not meet the framework's severity-and-permanence test in the operational terms the framework specifies, even where the substantive lived-experience of mental-health disability is operationally disabling.

The substantive policy response Part Three Section 3.2 develops — the social-model disability definition explicitly including episodic and fluctuating disability and operating independently of the categorical-program eligibility definitions — is calibrated to address this substantive structural barrier. The legislation's substantive protective function operates for persons with mental health disability regardless of whether the medical-model AISH-eligibility framework recognizes the disability in its

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

categorical-program-eligibility terms.

The distinction also matters for the substantive operational engagement with mental health disability within the K-12 system documented in Section 1.3a. The engineered repudiation pattern operates against children with mental-health-disability profiles in operationally specific ways. The behaviour-management approaches that produced the seclusion-room infrastructure (the substantive harm category Section 1.3a documents) operate disproportionately against children whose behavioural presentations are characterized as challenging within the medical-model framework: children with attention-deficit-hyperactivity-disorder, children with anxiety disorders that present as disruptive behaviour, children with depression that presents as withdrawal or non-compliance, children with trauma-related conditions, children whose neurodivergent profiles are characterized as behaviour problems, and children whose mental-health-disability profiles compound with other disability profiles in ways the medical-model framework characterizes as severe behaviour. The substantive social-model response operationalized in Part Three Section 3.4 (the inclusive-education baseline, the EA staffing-ratio provisions, the seclusion-room prohibition, the positive-behaviour-support framework, the trauma-informed-practice requirements) operates substantively against these patterns regardless of the medical-model characterization of the underlying conditions.

Mental Health and Addiction Across the Lifecycle Dimensions

Mental health disability and addiction operate across every lifecycle dimension Sections 1.1 through 1.6 documented. The substantive operational implications include:

Childhood and FSCD (Section 1.3). Children's mental health is substantively addressed within the FSCD program for children whose mental-health-disability profiles meet the FSCD eligibility criteria. The substantive operational reality is that many children with mental-health-disability profiles do not meet the FSCD eligibility criteria as currently administered, producing structural barriers to access for families whose children have substantive mental-health-disability needs. The substantive intersection with childhood-trauma-related conditions, including conditions arising from adverse childhood experiences documented in the Alberta substantive context, is operationally significant: the trauma-and-disability framework that the substantive policy response in Part Three Section 3.4 operationalizes addresses this intersection at the operational level.

K-12 Engineered Repudiation (Section 1.3a). The engineered repudiation pattern documented in Section 1.3a operates substantively against children with mental-health-disability profiles, as developed in the prior subsection. The substantive operational implication is that the funding-floor and EA-staffing-ratio provisions developed in Part Three Section 3.4 are operationally significant for the mental-health-disability dimension of the K-12 system. The substantive accommodation provisions, the trauma-informed-practice requirements, and the disability-affirming-practice requirements operate substantively against the medical-model characterizations that the engineered system has substantively reinforced.

Transition, PDD, and AISH (Section 1.4). The substantive AISH composition data establishes that mental illness disorders are the second-largest primary medical-condition category. The substantive AISH-to-ADAP transition documented in Section 1.4 affects the mental-health-disability population

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

substantively. The substantive ADAP eligibility criteria, the substantive monthly-benefit reduction (from one thousand nine hundred forty dollars to one thousand seven hundred forty dollars per month), the elimination of caseworker continuity, the elimination of appeal rights, and the structural narrowing of eligibility documented in Section 1.4 all operate substantively against the mental-health-disability population. The PDD program, by substantive design, addresses developmental disabilities specifically (cognitive-disability profiles diagnosed before age eighteen with documented IQ-and-adaptive-functioning thresholds); the PDD program does not substantively address mental-health-disability profiles that arise outside the developmental-disability framework, producing structural gaps in the categorical-program architecture for persons with adult-onset mental-health-disability profiles.

Caregivers (Section 1.5). The caregiver dimension operates substantively for caregivers of persons with mental-health-disability profiles. The substantive operational reality is that mental-health-disability caregiving is structurally distinct from physical-disability or cognitive-disability caregiving in operationally specific ways: the episodic-and-fluctuating nature of many mental-health-disability profiles produces structural caregiving demands that do not align with the categorical-program supports as currently administered; the substantive stigma associated with mental-health disability produces structural barriers to substantive caregiver support; and the substantive intersection of mental-health-disability caregiving with the broader mental-health-system architecture produces operational complications the substantive caregiver-support architecture does not adequately address.

Triage and WCB (Section 1.6). The WCB exception documented in Section 1.6 operates substantively for the mental-health-disability dimension in operationally specific ways. WCB-recognized mental-health conditions arising from workplace-related psychological injuries operate within the WCB substantive support framework, producing the operational distinction Section 1.6 identifies (substantive caseworker continuity, substantive vocational-rehabilitation supports, substantive community reintegration supports). The mental-health-disability population whose conditions did not arise from workplace injury operates within the substantively different AISH/ADAP architecture. The substantive triage question Section 1.6 raises — why the WCB-eligible-injured population receives substantively different supports than the AISH/ADAP population — operates with particular operational force in the mental-health-disability context, where the underlying mental-health condition may be substantively similar across the two populations but the substantive policy response is operationally distinct.

The cross-cutting operational implication is that mental health disability and addiction are not peripheral elements of the disability-policy architecture; they are substantive lifecycle dimensions that operate across the architecture's substantive content. The substantive case the blueprint develops accordingly addresses the mental-health-disability and addiction dimensions through the substantive standards and operational mechanisms developed in Part Three rather than through a separate categorical-program architecture.

The Substantive Alberta Record on Mental Health Disability

The substantive Alberta record on mental health disability includes four operationally significant dimensions: the AISH composition data and the substantive operational reality of the mental-health-disability AISH population, the *Mental Health Act* involuntary-confinement provisions and their substantive operational record, the documented inadequacy of community-based mental health

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

supports, and the substantive intersections with homelessness and incarceration.

AISH composition. As documented in the section's opening, mental illness disorders account for thirty-and-two-tenths percent of the September 2025 AISH caseload (twenty-three thousand nine hundred forty Albertans). The substantive historical record establishes that this proportion has been stable over time: a 2009 peer-reviewed study published in *Psychiatric Services* (Sayre, Black, Cromwell, and Wand, 2009, available at <https://psychiatryonline.org/doi/10.1176/ps.2009.60.5.686>) documented seventeen thousand one hundred thirty-eight Albertans with severe mental illness receiving AISH payments in 2005, with one hundred forty-two-and-eight-tenths million dollars in AISH supplemental income benefits and an additional sixty-four-and-four-tenths million dollars in federal CPP-Disability benefits. The 2005 baseline of one hundred forty-two-and-eight-tenths million dollars in AISH benefits to the severe-mental-illness population can be compared to the total provincial mental-health-services cost in the same year of four hundred five million dollars (per the Alberta Mental Health Board internal data documented in the same study), establishing that AISH income supports for the mental-health-disability population were operating at approximately thirty-five percent of the total provincial mental-health-services-expenditure level — a substantive structural-funding allocation that operates outside the clinical-treatment infrastructure.

The AISH legislative exclusion. The AISH eligibility framework, as documented at <https://www.alberta.ca/aish-eligibility>, excludes from eligibility persons living "in a correctional facility or some mental health facilities such as Alberta Hospital Edmonton." The exclusion is operationally significant. Alberta Hospital Edmonton operates as a major psychiatric facility within the provincial mental-health-system architecture; persons with mental-health-disability profiles whose substantive operational reality requires institutional psychiatric care are accordingly excluded from AISH benefits during their institutional stays. The structural-policy-rationale for the exclusion is that institutional psychiatric care is provided through the provincial-government health-services budget, eliminating the substantive operational need for income-support during the institutional stay; the substantive operational consequence, however, is that persons whose mental-health-disability profiles produce intermittent or extended psychiatric-institutional stays face structural disruptions to their AISH benefits and to the substantive housing, healthcare, and community-support arrangements that AISH benefits support during their non-institutional periods. The substantive intersection with the *Mental Health Act* involuntary-confinement framework (developed in the next paragraph) compounds this operational pattern.

The *Mental Health Act* of Alberta. The *Mental Health Act* (R.S.A. 2000, c. M-13) provides for involuntary admission and treatment of persons with mental disorders meeting the substantive operational criteria the Act specifies. The substantive operational provisions include involuntary admission to designated mental-health facilities for assessment and treatment where the person is determined to pose a substantive risk of harm to themselves or others or to be substantively unable to care for themselves due to the mental disorder. The Act's substantive operational provisions establish the Alberta Mental Health Patient Advocate and the substantive review-board framework through which involuntary-admission decisions are reviewed. The substantive parallel to the Section 1.3a seclusion-room analysis is operationally significant: the *Mental Health Act* operates the substantive provincial framework for involuntary confinement of persons with mental-health-disability profiles, just as Ministerial Order #042/2019 operates the substantive provincial framework for involuntary confinement of children with

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

developmental-and-cognitive-disability profiles in school settings. The substantive parallel raises substantive questions about the substantive consistency of the provincial-government's involuntary-confinement framework across populations: where the substantive harm category Section 1.3a documents in the school context is substantively analogous to the substantive harm category produced by *Mental Health Act* involuntary-confinement, the substantive policy responses should be operationally consistent. The substantive case the blueprint develops accordingly engages the involuntary-confinement framework substantively at the substantive standards level (Part Three Section 3.3 Area 6 health-area standard, with the Article 12 supported-decision-making framework operationalizing the substantive alternative).

Community-based mental-health supports. The substantive Alberta record on community-based mental-health supports establishes structural inadequacy at the operational level. The documented gaps include: insufficient community-based mental-health-services capacity relative to the substantive population needs; structural barriers to access for the mental-health-disability population including substantive wait-times for community-based services, substantive geographic gaps in service availability particularly in northern, rural, and remote Alberta (developed in Section 1.8 forthcoming), and substantive cost barriers for services not covered under the provincial Health Care Insurance Plan; the documented inadequacy of crisis-response infrastructure including the substantive operational reality that the Alberta crisis-response architecture relies substantively on emergency-department and law-enforcement frontline-response that produces operational outcomes documented to be substantively inadequate to the substantive needs the population presents; and the documented inadequacy of supported-housing infrastructure for the mental-health-disability population, producing the operational reality that substantial portions of the mental-health-disability population face structural homelessness or housing-precarity.

Homelessness and incarceration intersections. The substantive Alberta record on the intersection of mental health disability with homelessness and incarceration is operationally documented. A 2024 Homeward Trust report (cited in <https://globalnews.ca/news/11009443/alberta-opioid-data-overdoses-2025/>) documented over four thousand five hundred persons living rough in Edmonton; the *Edmonton Journal* reported one thousand four hundred twenty-nine shelter spaces in the same year; the provincial-government homeless-shelter tracking system as of February 2025 showed two thousand forty-six shelter spaces in Edmonton, with ninety-three percent in operational use. The substantive operational reality is that a substantial portion of the homeless population in major Alberta urban centres has documented mental-health-disability profiles, and that the substantive structural-housing inadequacy compounds with the substantive structural-mental-health-supports inadequacy to produce the operational outcomes documented (including, per Thomson cited in the same source, that homeless individuals face approximately two hundred to one thousand times higher risk of death from drug poisoning than housed people). The substantive intersection with incarceration operates analogously: the substantive Alberta correctional-facility population includes substantial portions with mental-health-disability profiles, and the substantive structural-mental-health-supports inadequacy compounds with the substantive structural-criminalization patterns to produce the operational outcomes documented.

The Substantive Alberta Record on Addiction and Substance Use

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

The substantive Alberta record on addiction and substance use is operationally documented through the Alberta Substance Use Surveillance System and the federal Public Health Agency of Canada national surveillance data. The substantive operational reality is that Alberta has experienced a sustained drug-poisoning crisis over the period from 2016 to the present, with substantive operational variations across years and across regions within the province.

The substantive scale of the crisis. Public Health Agency of Canada national surveillance data establishes that fifty-five thousand thirty-two apparent opioid toxicity deaths were reported nationally between January 2016 and September 2025, with four thousand one hundred sixty-two apparent opioid toxicity deaths reported nationally for the January-to-September 2025 reporting period (96 percent accidental). Source: <https://health-infobase.canada.ca/substance-related-harms/opioids-stimulants/>. The Alberta-specific substantive record establishes that 2023 produced over one thousand eight hundred opioid-related deaths in Alberta (a record high since national surveillance began in 2016); 2024 produced one thousand one hundred eighty-nine opioid-related deaths (a substantive decline from 2023); and the early-2025 period produced operationally significant variation with March 2025 producing eighty-seven opioid-related deaths in the Edmonton zone alone (the highest single-month total in any Alberta zone since tracking began publicly in 2016) and one hundred seventeen opioid-related deaths province-wide. Sources: <https://www.cbc.ca/news/canada/edmonton/edmonton-drug-poisoning-opioid-1.7575814>; <https://globalnews.ca/news/11272959/edmonton-opioid-deaths-alberta-carfentanil/>.

The carfentanil escalation. The substantive Alberta record establishes that carfentanil — an opioid approximately one hundred times more potent than fentanyl and ten thousand times more potent than morphine — has substantively escalated in the Alberta drug supply over the period from 2024 to 2025. Carfentanil involvement in Alberta opioid deaths increased from approximately ten percent in 2024 to sixty-eight percent for the January-to-May 2025 period; in the Edmonton zone specifically, carfentanil involvement increased from seventeen percent in 2024 to seventy-eight percent in the same 2025 period; and Edmonton accounted for approximately eighty-eight percent of all Alberta carfentanil-related deaths during the January-to-May 2025 period. Source: <https://www.stalbertgazette.com/beyond-local/opioid-related-deaths-are-on-the-rise-but-numbers-in-one-alberta-region-suggests-a-different-story-11214865>.

The shift from harm-reduction to recovery-focused policy. The substantive provincial-government policy direction over the period from 2019 forward has shifted substantively from harm-reduction-oriented approaches to recovery-focused approaches. The substantive operational changes include: the closure of supervised consumption sites including Lethbridge in 2020 and Red Deer in 2025; the implementation of identification requirements for supervised consumption site users in 2022, eliminating substantive operational anonymity protections; the substantive provincial-government investment in residential-treatment infrastructure (over ten thousand publicly-funded treatment spaces added during the period, the establishment of the Recovery Alberta provincial health agency in 2024); and the substantive provincial-government policy framing that characterizes harm-reduction approaches as substantively problematic and recovery-focused approaches as substantively appropriate to the operational reality. Source: <https://theconversation.com/drop-in-drug-poisoning-deaths-does-not-necessarily-indicate-alberta-recovery-model-is-working-239722>. The substantive academic-and-professional engagement with this policy direction includes substantial documented criticism that harm-reduction approaches are evidence-supported in the public-health literature and that the provincial-government policy direction is

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

substantively at odds with the established public-health evidence base.

The Opioid Dependency Program enrollment trajectory. The Alberta Opioid Dependency Program, which provides methadone or buprenorphine/naloxone (Suboxone) treatment to persons with documented opioid dependence, has experienced substantial enrollment growth over the period from 2018 to the present. ODP enrollment grew from three thousand two hundred eighty-eight clients in 2018 to twelve thousand two hundred ninety-nine clients in 2024, with six thousand seven hundred eighty-nine clients enrolled at the time of the September 2025 reporting (consistent with the year-over-year growth pattern). Source: <https://www.stalbertgazette.com/beyond-local/opioid-related-deaths-are-on-the-rise-but-numbers-in-one-alberta-region-suggests-a-different-story-11214865>. The substantive operational implication is that the substantive demand for opioid-dependence treatment in Alberta is substantively documented and operating at substantial scale.

The *Compassionate Intervention Act* and the Involuntary-Treatment Framework

The substantive provincial-government policy response to the addiction-and-mental-health intersection received substantive operational expression through the *Compassionate Intervention Act* (Bill 53), introduced in the Legislative Assembly on April 15, 2025, and receiving Royal Assent on May 15, 2025. Source: <https://www.cbc.ca/news/canada/edmonton/alberta-introduces-controversial-involuntary-addiction-s-treatment-bill-1.7511051>; <https://www.alberta.ca/compassionate-intervention>.

The substantive legislative content. The *Compassionate Intervention Act* establishes a substantive provincial framework for involuntary treatment of persons with severe substance-use or addiction whose conduct is determined to be likely to cause substantial harm to themselves or others within a reasonable time. The substantive provisions include: authority for adult family members, guardians, healthcare professionals, police officers, and peace officers to apply for an assessment order; a three-person Compassionate Intervention Commission (consisting of a lawyer, a physician, and a member of the public) appointed by the provincial-government to review applications and issue legally-binding treatment decisions; a seventy-two-hour assessment-and-detoxification period during which the person may be involuntarily detained for assessment, with hearings before the full Commission required within the seventy-two-hour period; secure treatment orders for up to three months in designated compassionate-intervention treatment facilities; community-based treatment orders for up to six months; and care-plan reviews every three weeks during the treatment period. The Act replaces the 2006 *Protection of Children Abusing Drugs Act* (PChAD) for youth, providing the substantive operational framework for involuntary youth-addiction-treatment.

The substantive infrastructure investment. The substantive provincial-government investment in compassionate-intervention infrastructure has been progressively announced over 2025 and 2026. Budget 2025 allocated one hundred eighty million dollars over three years; Budget 2026 increased the allocation to two hundred ninety-one million dollars over three years (2026-2029) for the construction of two one-hundred-fifty-bed compassionate-intervention centres in Edmonton and Calgary, with construction beginning in 2026 and the centres expected to open by 2029. Recovery Alberta operates as

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

the provincial health agency administering the framework. Approximately one hundred secure beds are being established in existing facilities and hospital units to support the implementation phase. Source: <https://www.alberta.ca/compassionate-intervention>.

The substantive *Mental Health Act* relationship. The *Compassionate Intervention Act* expands the substantive provincial involuntary-treatment framework beyond the existing *Mental Health Act* infrastructure. The substantive *Mental Health Act* infrastructure operates against persons with mental disorders meeting the substantive impaired-capacity test; the *Compassionate Intervention Act* operates against persons with severe substance-use or addiction even where the substantive capacity test is met. The substantive academic-and-professional engagement with this expansion includes substantial documented concern that the substantive expansion lowers the substantive evidentiary threshold for involuntary detention and treatment beyond the substantive *Mental Health Act* framework. Source: <https://www.fieldlaw.com/insights/publication/Compassion-vs-Coercion-Regulatory-Impacts-of-Bill-53>.

The substantive Charter implications. The substantive Charter implications of the *Compassionate Intervention Act* are operationally documented through professional-and-academic engagement. Eric Adams, a constitutional-law professor at the University of Alberta, has stated that the legislation is expected to face Charter challenge once it becomes operational, given its substantive interference with Charter Section 7 rights (life, liberty, and security of the person) and Charter Section 9 rights (freedom from arbitrary detention). The Canadian Bar Association Alberta branch issued a May 2025 letter raising substantive legal concerns including the substantive overlap with existing legislation (the *Personal Directives Act*, the *Adult Guardianship and Trusteeship Act*, the *Youth Criminal Justice Act*), the substantive procedural-fairness questions including the substantive ability of the "client" to participate in the assessment process and the substantive presumption of capacity questions, and the substantive consent-and-treatment provisions including authorization of medication administration without consent. Source:

<https://cba-alberta.org/our-impact/submissions/may-2025-letter-re-compassionate-intervention-act/>. The Canadian Medical Association Journal published a substantive ethical analysis applying the Kass framework for public-health interventions and concluding that the *Compassionate Intervention Act* presents substantive risks across all three substantive Kass-framework domains (privacy, liberty, and justice), with substantive operational consequences including the potential for the substantive disproportionate targeting of structurally marginalized populations including First Nations, Inuit, and Métis individuals. Source: <https://www.cmaj.ca/content/197/40/E1346>.

The substantive parallel to the engineered repudiation pattern. The *Compassionate Intervention Act* operates within the substantive provincial-government policy direction that the Section 1.3a engineered-repudiation pattern is part of. The substantive operational parallel is operationally significant: where Section 1.3a documents the substantive provincial-government use of involuntary-confinement infrastructure (the seclusion-room expansion, Ministerial Order #042/2019) within the K-12 system to address substantive accommodation challenges that the substantive structural-resourcing inadequacy produces, the *Compassionate Intervention Act* documents the substantive provincial-government use of involuntary-confinement infrastructure within the addiction-and-mental-health system to address substantive accommodation challenges that the substantive structural-resourcing inadequacy produces (the documented inadequacy of community-based supports, the documented inadequacy of

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

voluntary-treatment infrastructure, the documented inadequacy of supported-housing infrastructure). The substantive structural-policy pattern is operationally consistent across the two substantive contexts: where the substantive structural-resourcing inadequacy produces operational accommodation challenges, the substantive provincial-government policy response operates through expanded involuntary-confinement infrastructure rather than through expanded substantive-resourcing of voluntary-and-community-based alternatives.

The substantive case the blueprint develops engages this pattern substantively. Part Three Section 3.3 Area 6 (health-area standard) operationalizes Article 25 of the United Nations *Convention on the Rights of Persons with Disabilities* (right to highest attainable standard of health without discrimination on the basis of disability), Article 12 (equal recognition before the law, including supported decision-making frameworks), and Article 19 (living independently and being included in the community). Part Three Section 3.4 (education-area standard) operationalizes Article 24 substantively, including the substantive seclusion-room prohibition that addresses the K-12 expression of the involuntary-confinement pattern. The substantive accessibility-standards architecture the blueprint develops operates against the involuntary-confinement-as-policy-response pattern at the substantive level the international human rights framework requires.

The Co-Occurrence of Mental Health and Addiction

The substantive operational reality is that mental-health disability and addiction are substantively co-occurring conditions for a substantial portion of the population the legislation is intended to serve. The peer-reviewed academic literature establishes that substance-use disorders co-occur with mental-health-disability profiles at substantively higher rates than would be expected by chance, and that the substantive operational interaction between the two conditions produces complications that operationally distinguish co-occurring presentations from either condition operating alone. The substantive Alberta record reflects this co-occurrence at the operational level: substantial portions of the AISH mental-illness-disorders population have co-occurring substance-use disorder profiles, substantial portions of the *Compassionate Intervention Act*-eligible population have co-occurring mental-health-disability profiles, and substantial portions of the homeless population in major Alberta urban centres have both mental-health-disability and substance-use-disorder profiles compounding with structural housing inadequacy.

The substantive policy implication is that the disability-policy architecture cannot operate substantively distinct policy responses to mental-health disability and to addiction without producing substantive operational gaps for the co-occurring population. The substantive case the blueprint develops accordingly engages the two dimensions through integrated substantive standards rather than through separate categorical-program architectures. The Part Three Section 3.3 Area 6 substantive content operationalizes the substantive integrated-care framework that the international human rights framework supports.

The substantive operational complications of the co-occurring profile include: the substantive medical-model treatment infrastructure that often operates the two conditions through structurally separate clinical pathways, producing operational coordination gaps; the substantive AISH-eligibility framework that requires medical certification for the qualifying condition, producing structural ambiguity for persons whose

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

mental-health-disability profile may be characterized as primarily mental-health-disability or primarily substance-use-disorder depending on the substantive professional-clinical judgment of the certifying medical professional; the substantive *Compassionate Intervention Act* framework that operates against the substance-use-disorder dimension while the substantive *Mental Health Act* framework operates against the mental-health-disability dimension, producing substantive operational complications for the co-occurring population whose substantive presentation may be characterized through either framework; and the substantive housing-and-community-supports inadequacy that operates against the co-occurring population at the substantive operational level.

What Section 1.7 Establishes

The substantive lifecycle dimension of mental health disability and addiction is now documented at the depth required for the substantive case the blueprint develops. The substantive content establishes seven substantive findings.

First, mental health disability is the second-largest primary medical-condition category among the AISH population, accounting for thirty-and-two-tenths percent of the September 2025 caseload (twenty-three thousand nine hundred forty Albertans). The substantive AISH-to-ADAP transition documented in Section 1.4 affects this population in operationally specific ways the prior sections do not address.

Second, the substantive distinction between mental illness as a medical condition and mental health disability as a dimension of social participation is operationally significant for the substantive case. The medical-model AISH-eligibility framework produces structural barriers to access for persons with substantive mental-health-disability profiles whose disability does not meet the framework's severity-and-permanence test in the operational terms the framework specifies. The Part Three Section 3.2 social-model disability definition explicitly including episodic and fluctuating disability operationalizes the substantive policy response.

Third, the substantive Alberta record on mental health disability includes the *Mental Health Act* involuntary-confinement provisions that operate as the substantive parallel to the Section 1.3a seclusion-room infrastructure, the AISH legislative exclusion of correctional facilities and certain mental-health facilities including Alberta Hospital Edmonton, the documented inadequacy of community-based mental-health supports, and the substantive intersections with homelessness and incarceration.

Fourth, the substantive Alberta record on addiction includes the documented opioid-and-stimulant toxicity crisis (over one thousand eight hundred deaths in 2023, one thousand one hundred eighty-nine deaths in 2024, the March 2025 Edmonton-zone single-month record of eighty-seven deaths), the substantive carfentanil escalation in the drug supply (sixty-eight percent involvement provincially, seventy-eight percent in Edmonton, January-May 2025), and the substantive provincial-government policy direction shifting from harm-reduction toward recovery-focused approaches (supervised-consumption-site closures, identification requirements, substantial residential-treatment infrastructure investment).

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Fifth, the *Compassionate Intervention Act* (Bill 53, Royal Assent May 15, 2025) operationalizes the substantive provincial-government involuntary-treatment framework for severe substance-use and addiction, expanding the substantive *Mental Health Act* framework beyond the impaired-capacity test to substance-use-disorder presentations even where capacity is intact. The substantive Charter implications, the substantive academic-and-professional concerns documented through the Canadian Medical Association Journal, the Canadian Bar Association Alberta, the Alberta Medical Association, and the substantive engagement of constitutional-law expertise establish that the substantive framework operates within ongoing substantive legal-and-ethical contestation.

Sixth, the substantive parallel between the K-12 engineered-repudiation pattern documented in Section 1.3a and the substantive provincial-government use of involuntary-confinement infrastructure under the *Compassionate Intervention Act* operates as substantive operational evidence of a consistent provincial-government policy pattern: where structural-resourcing inadequacy produces operational accommodation challenges, the substantive policy response operates through expanded involuntary-confinement infrastructure rather than through expanded substantive-resourcing of voluntary-and-community-based alternatives. The substantive accessibility-standards architecture the blueprint develops in Part Three operates against this pattern at the substantive level the international human rights framework requires.

Seventh, the substantive co-occurrence of mental health disability and addiction within substantial portions of the population the legislation is intended to serve produces substantive operational complications that integrated substantive standards (Part Three Section 3.3 Area 6) address more substantively than separate categorical-program architectures. The substantive policy implication is that the disability-policy architecture cannot operate substantively distinct policy responses to mental-health disability and to addiction without producing substantive operational gaps for the co-occurring population.

The substantive content of Section 1.7 extends the lifecycle frame established in Section 1.2 to address the mental-health-and-addiction dimension at the depth the documented substantive Alberta record requires. The substantive case the blueprint develops in Parts Two and Three operates substantively against the documented patterns through the substantive accessibility-standards architecture, the substantive Article 4(3) consultation requirements, and the substantive structural-resourcing mechanisms that prevent the documented patterns from being reproduced under different operational labels.

The next section turns to the substantive lifecycle dimension of rural, remote, and northern Alberta, addressing the operationally distinct disability experience that the geographic-and-cultural distinctions within Alberta produce.